Behavioral Health Agency Inspection Report

Department of Health
P.O. Box 47874, Olympia, WA 98504-7874

January 29, 2025

Comprehensive Healthcare - Two Rivers Landing, 504 S 3rd Ave, Yakima, WA. 98902-3543		Edie Dibble
Agency Name and Address		Administrator
ONGOING - ROUTINE	10/14/24-11/14/24	GLD03/DEC13
Inspection Type	Inspection Onsite Dates	Inspector
X2024-800	BHA.FS.60873007	
Inspection Number	License Number	

Please note that the deficiencies/violations/observations noted in this report are not all-inclusive, but rather were deficiencies/violations/observations that were observed or discovered during the on-site inspection.

Deficiency Number and Rule Reference	Observation Findings	Plan of Correction

0695 Personnel-Agency record requirements
WAC 246-341-0510(1)(iii) A behavioral health agency must maintain a personnel record for each person employed by the agency. (1) The personnel record must contain all of the following: (c) A record of an orientation to the agency within 90 days of hire that includes all of the following: (iii) The process for resolving client concerns.
	Based on personnel record review, policy, and procedure (P&P) review, and interview it was determined the agency failed to maintain a personnel record for each employee that contained a record of orientation within 90 days of hire that included the process for resolving client complaints and grievances.
	
	Failure to meet this WAC standard places clients at risk due to staff not having an understanding of client's rights to file a complaint or grievance or the steps to follow in the case of a complaint or grievance filed by the client.
	
	Findings included:
		How the Deficiency Will Be Corrected:
		• The Client Right attestation form has been updated to reflect current client rights as well as additional references and information regarding the following policies:
		  ○ Client Advocate (RI-007)
		  ○ Client Concerns & Grievances (RI-001)
		• All new employees will sign this updated form to attest that they understand the client rights and where to access additional information about

	1. Review of the agency P&Ps submitted with various titles and policy numbers for “HR”, failed to address orientation, specific required trainings including the process for resolving client concerns. 2.Review of the personnel training records for Staff, A, Registered Nurse (RN), Staff B, Behavioral Health Associate (BHA) and Staff C, Behavioral Health Associate (BHA) showed no orientation training of the process for resolving client concerns. 3. During an interview on 10/14/24 at 10:26 AM with Staff H, Human Resources (HR), they indicated this topic may be a part of the “employee attestation checklist” completed when they attend orientation training.	the Client Advocate's role if needed. • The Quality Improvement onboarding training has been updated to include more detailed information on the Client Advocate and their role within the organization in addressing client concerns, complaints, or questions. This training is conducted during the first week of the employee’s onboarding. • An article on the Client Advocate will appear in our internal Connections employee eNewsletter in February to remind all staff of the Advocate’s role and functions within the organization. **Who is responsible:** • Senior Vice President of Human Resources • Quality Improvement Coordinator • Director of Marketing **Date of Correction:** • Onboarding Training: Completed as of 2/10/25. • Form implementation: By 2/28/25. • Article: By 2/28/25.

		Monitors put in place to assure continuing compliance: • Onboarding checklist for each new hire is audited daily until all items are completed, which includes the review and signature of the updated Client Rights Attestation Form.
3535 Residential and inpatient mental health services WAC 246-341-1118(1)(a) (1) An agency providing residential and inpatient mental health services must develop and implement an individualized annual training plan for agency staff members, to include at least: (a) Least restrictive alternative options available in the community and how to access them.	Based on personnel record review and interview it was determined the agency failed to implement an annual individualized training plan for agency staff members that included the least restrictive options available in the community and how to access them. Failure to meet this WAC standard may harm clients and places the client at a disadvantage, creating barriers for knowing and accessing the least restrictive options in the community that could assist in recovery goals. Findings included: 1. Review of P&Ps submitted failed to address orientation training and staff training plans. 2. Review of the personnel record for Staff A, RN, date of hire 06/05/23, showed no LRA training completed until ProACT training on 10/11/24 and showed no annual training plan in the record. 3. Review of the personnel record for Staff B, BHA, date of hire 07/10/23, showed no LRA completed until ProACT training on 12/20/23.	**How the Deficiency Will Be Corrected:** • Onboarding and annual training plans are in place for all clinical and non-clinical staff. These "Training Plans" in Relias, our online training platform, contain multiple individual trainings. Training plans exist with specific categories for staff including RNs and BHA. • The current LRA training in Relias is not in the current Training Plans. • A custom training on LRAs that focuses specifically on the needs of residential and inpatient staff will be developed and added to the current applicable Training Plans in Relias. **Who is responsible:** • Senior Vice President of Human Resources • Senior Director for Quality Improvement

	4. Review of the personnel record for Staff B, BHA, date of hire 03/06/23, showed completed ProACT training on 06/04/23, with no annual training plan. 5. During an interview on 10/14/24 at 10:26 AM with Staff H, HR, they indicated they do not know why the training or annual evaluation would not have been completed. They did indicate the staff may have had to wait until the agency had a trainer scheduled to provide the training.	**Date of Correction:** • The custom LRA training will be added to the applicable Training Plans by 3/31/25. **Monitors put in place to assure continuing compliance:** Relias (our online training system) will automatically send supervisors pending and late training completions.

3540 Residential and inpatient mental health services
WAC 246-341-1118(1)(b) (1) An agency providing residential and inpatient mental health services must develop and implement an individualized annual training plan for agency staff members, to include at least: (b) Methods of providing individualized treatment.
	Based on personnel record review and interview it was determined the agency failed to implement an annual individualized training plan for agency staff members that included methods of providing individualized treatment.
	Failure to provide staff with an annual training plan including how to provide a more specific individualized treatment may create a barrier for the client's recovery and cause harm.
	
	Findings included:
	
	1. Review of P&Ps submitted failed to address orientation training and staff training plans.
	
	2. Review of the personnel record for Staff A, RN, date of hire 06/05/23, showed no LRA training completed until ProACT training on 10/11/24 and showed no annual training plan in the record.
	
	3. Review of the personnel record for Staff B, BHA, date of hire 07/10/23, showed no LRA completed until ProACT training on 12/20/23.
		How the Deficiency Will Be Corrected:
		• Individualized training plans are in place for inpatient & residential positions.
		• Inpatient & Residential nurses, and other medical providers, complete the following trainings during onboarding:
		  ○ Boundaries in the Treatment Relationship
		  ○ Legal and Ethical Standards for Behavioral Health Professionals
		  ○ Recovery Principles and Practices in Behavioral Health Treatment
		  ○ Integration of Primary and Behavioral Healthcare
		  ○ Strategies for Behavioral Health Professionals to Manage Chronic Stress

	4. Review of the personnel record for Staff B, BHA, date of hire 03/06/23, showed completed ProACT training on 06/04/23, with no annual training plan. 5. During an interview on 10/14/24 at 10:26 AM with Staff H, HR, they indicated they do not know why the training or annual evaluation would not have been completed. They did indicate the staff may have had to wait until the agency had a trainer scheduled to provide the training	○ The Biopsychosocial Model of Substance-Related and Addictive Disorders • The BHA Training Plan includes all the courses listed above plus: ○ Supporting Integrated Care through Case and Care Management ○ Psychiatric Medications: An Overview for Paraprofessionals • A customized training for Inpatient & Residential staff on LRAs and LRA Management will be added to all Residential and Inpatient Training Plans in Relias. • Annual training plans include training on the following topics: cultural competence and humility, infection control and prevention, bloodborne pathogens, trauma informed treatment principles, abuse and neglect prevention and intervention, suicide prevention and intervention, ethics and the law, corporate compliance, essentials of HIPSS, harassment in the workplace, and ensuring a safe work environment. • Timely completion of ProACT training is addressed below. **Who is responsible:**

		• Senior Vice President for Human Resources • Senior Director for Quality Improvement **Date of Correction:** • Training plans with the new customized LRA training added will be updated by 3/31/25. **Monitors put in place to assure continuing compliance:** • Relias (online training/reporting system) will automatically send to the Directors/Supervisors prior to annual due date
3545 Residential and inpatient mental health services WAC 246-341-1118(1)(c) (1) An agency providing residential and inpatient mental health services must develop and implement an individualized annual training plan for agency staff members, to include at least: (c) De-escalation training and management of assaultive and self-destructive behaviors, including proper and safe use of seclusion and restraint procedures.	Based on personnel record review and interview it was determined the agency failed to implement an annual individualized training plan for agency staff members that included de-escalation training and management of assaultive and self-destructive behaviors, including proper and safe use of seclusion and restraint procedures. Failure to meet this WAC standard and provide training to staff on de-escalation and assaultive and self-destructive behaviors, including proper and safe use of seclusion and restraint procedures may violate client civil rights and have a potential for increased harm to the client. Findings included: 1. Review of P&Ps submitted failed to address orientation training and staff training plans.	**How the Deficiency Will Be Corrected:** • Training for all staff includes an Annual Training on "Ensuring a Safe Work Environment." This training meets the requirements of RCW 49.19.030 for health care workers and always includes information on de-escalation skills. • All new employees complete a "Preventing and De-Escalating Crisis Situations" training within 30 days of employment. • In addition, Residential and Inpatient staff receive initial ProACT training as well as annual boosters.

	2. Review of the personnel record for Staff A, RN, date of hire 06/05/23, showed no LRA training completed until ProACT training on 10/11/24 and showed no annual training plan in the record. 3. Review of the personnel record for Staff B, BHA, date of hire 07/10/23, showed no LRA completed until ProACT training on 12/20/23. 4. Review of the personnel record for Staff B, BHA, date of hire 03/06/23, showed completed ProACT training on 06/04/23, with no annual training plan. 5. During an interview on 10/14/24 at 10:26 AM with Staff H, HR, they indicated they do not know why the training or annual evaluation would not have been completed. They did indicate the staff may have had to wait until the agency had a trainer scheduled to provide the training.	• HR will ensure that the initial ProACT training is scheduled on date of hire, to be completed within 90-days of hire. • New evening time ProACT trainings have been established to accommodate night shift staff. • ProACT training is now on a regular calendar and is offered more frequently. **Who is responsible:** • Senior Vice President for Human Resources • VP for Residential and Inpatient services and acting Program Manager for Two Rivers Landing **Date of Correction:** • Completed as of 2/10/25. **Monitors put in place to assure continuing compliance:** • Relias (our online training system) will automatically send supervisors pending and overdue training reminders on a weekly basis.

Introduction

We require that you submit a plan of correction for each deficiency listed on the inspection report form. Your plan of correction must be submitted to the DOH within ten business days of receipt of the list of deficiencies.

Descriptive Content

Your plan of correction must provide a step-by-step description of the methods to correct each deficient practice to prevent recurrence and provide information that ensures the intent of the regulation is met.

An acceptable plan of correction must contain the following elements:

- The plan of correcting the specific deficiency.
- The procedure for implementing the acceptable plan of correction for the specific deficiency cited.
- The monitoring procedure to ensure that the plan of correction is effective, and that specific deficiency cited remains corrected and/or in compliance with the regulatory requirements.
- The title of the person responsible for implementing the acceptable plan of correction.

Simply stating that a deficiency has been "corrected" is not acceptable. If a deficiency has already been corrected, the plan of correction must include the following:

- How the deficiency was corrected,
- The completion date (date the correction was accomplished),
- How the plan of correction will prevent possible recurrence of the deficiency.

Completion Dates

The POC must include a completion date that is realistic and coincides with the amount of time your facility will need to correct the deficiency. Direct care issues must be corrected immediately and monitored appropriately. Some deficiencies may require a staged plan to accomplish total correction. Deficiencies that require bids, remodeling, replacement of equipment, etc., may need more time to accomplish correction; the target completion date, however, should be within a reasonable and mutually agreeable timeframe.

Continued Monitoring

Each plan of correction must indicate the appropriate person, either by position or title, who will be responsible for monitoring the correction of the deficiency to prevent recurrence.

Checklist:

- Before submitting your plan of correction, please use the checklist below to prevent delays.
- Have you provided a plan of correction for each deficiency listed?
- Does each plan of correction show a completion date of when the deficiency will be corrected?
- Is each plan descriptive as to how the correction will be accomplished?
- Have you indicated what staff position will monitor the correction of each deficiency?
- If you included any attachments, have they been identified with the corresponding deficiency number or identified with the page number to which they are associated?

Your plan of correction will be returned to you for proper completion if not filled out according to these guidelines.

Note: Failure to submit an acceptable plan of correction may result in enforcement action.

Approval of POC
Your submitted POC will be reviewed for adequacy by DOH. If your POC does not adequately address the deficiencies in your inspection report, you will be sent a letter detailing why your POC was not accepted.

Questions?
Please review the cited regulation first. If you need clarification or have questions about deficiencies you must contact the inspector who conducted the onsite inspection, or you may contact the supervisor.

STATE OF WASHINGTON

DEPARTMENT OF HEALTH

2/18/2025

Edie Dibble
Two Rivers Landing
PO Box 959
Yakima, WA 98907-0959

Re: Exam Number: X2024-800
License Number: BHA.FS.60873007
Approved Plan of Correction
Date(s) of Inspection: 10/2/24-11/26/24

Dear Administrator:

This letter is to inform you that after careful review of the Plan of Correction (POC) you submitted for the exam recently conducted at your facility, the Department has determined that the POC is approved.

Based on the scope and severity of the deficiencies listed in your statement of deficiency report, the Department **will not** conduct an announced follow-up compliance visit to verify that all deficiencies have been corrected.

Reviewer:
Department of Health
HSQA/Office of Health Systems Oversight
PO Box 47874
Olympia, Washington 98504-7874